Borderline and Other Personality Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

A personality disorder is a long-standing, inflexible pattern of thinking, feeling, relating, and impulse control that differs markedly from cultural expectations, begins by adolescence or early adulthood, and causes distress or impairment across many situations (American Psychiatric Association [APA], 2022). DSM-5-TR groups ten personality disorders into three clusters: **A** (odd or eccentric: paranoid, schizoid, schizotypal), **B** (dramatic or erratic: antisocial, borderline, histrionic, narcissistic), and **C** (anxious or fearful: avoidant, dependent, obsessive-compulsive).

How common: across 46 studies in 21 countries, 7.8% of adults had at least one personality disorder (Winsper et al., 2020).

Borderline personality disorder (BPD) is the most studied and the focus of this review. It involves instability in emotions, relationships, and self-image, with impulsivity and often self-harm (Gunderson et al., 2018).

Hope for recovery: over 16 years, 78% to 99% of patients with BPD achieved symptom remission, though fewer (40% to 60%) achieved full recovery with good social and work functioning (Zanarini et al., 2012). BPD is not a life sentence.

2. Core symptoms to watch for

DSM-5-TR criteria for BPD, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive pattern, beginning by early adulthood, with **5 or more** of:

- Frantic efforts to avoid real or imagined **abandonment**.
- Unstable, intense **relationships** that swing between idealizing and devaluing.
- Unstable **self-image** or sense of identity, and chronic feelings of **emptiness**.
- **Impulsivity** in at least two self-damaging areas (such as spending, sex, substances, reckless driving, binge eating).
- Recurrent **suicidal behavior, gestures, threats, or self-harm**.
- Intense, reactive **mood swings** lasting hours to a few days, and intense, hard-to-control **anger**.
- Brief, stress-related **paranoid thinking or dissociation**.

Screening: the McLean Screening Instrument (MSI-BPD) was developed as a brief self-report screen for BPD (Zanarini et al., 2003). Diagnosis requires a full clinical assessment.

Suicide risk: across 8 studies of 1,179 patients with BPD, 94 died by suicide, far above the general-population rate (Pompili et al., 2005). Assess self-harm and suicide risk at every stage, and build a safety plan.

When counselors must refer (general clinical consensus): BPD needs a structured, specialist-supported approach. Refer for psychiatric evaluation for suicidality, serious self-harm, psychosis, or substance dependence, and seek consultation when treating BPD.

3. What works: treatment and the evidence

Bottom line for class: structured psychotherapy, especially dialectical behavior therapy (DBT), helps; well-organized general psychiatric care can work about as well. Medication does not treat BPD itself.

Evidence overview (honest effect sizes):

- **Psychotherapy overall:** in a Cochrane review of 75 trials (4,507 people), psychotherapy reduced BPD severity compared with usual care (SMD −0.52, moderate-quality evidence), and modestly reduced self-harm and suicide-related outcomes (Storebø et al., 2020). Another meta-analysis of 33 trials found small-to-moderate effects; DBT and psychodynamic approaches had the clearest support, and bias inflated many results (Cristea et al., 2017).
- **DBT** (a cognitive-behavioral treatment teaching mindfulness, emotion regulation, distress tolerance, and relationship skills): compared with expert community therapists, DBT halved suicide attempts and reduced dropout and hospital use over 2 years (Linehan et al., 2006). However, **general psychiatric management** did about as well as DBT in a 1-year trial (McMain et al., 2009).

- **Standard CBT:** adding CBT to usual care did not reduce the combined rate of suicidal acts, hospital admissions, and emergency visits, though suicidal acts alone did decline (Davidson et al., 2006).
- **Other personality disorders:** among 323 patients, schema therapy (which targets long-standing core beliefs about self and others) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014).
- **Medication (referral notes):** in 46 trials, no medication improved BPD's primary outcomes more than placebo (Stoffers-Winterling et al., 2022). Medication may still treat co-occurring depression, anxiety, or psychosis.

Cognitive-behavioral work with BPD: how it is used (Linehan et al., 2006; Bamelis et al., 2014):

- **Validate first, then change.** Accept the person's pain while building new skills.
- **Build skills:** mindfulness, naming and regulating emotions, tolerating distress without self-harm, and asking for needs without damaging relationships.
- **Identify and challenge core beliefs** such as "I am unlovable," "Everyone will abandon me," "I am bad," and "I can't stand this feeling."

4. Religion, spirituality, and Christian integration: what the evidence shows

Spirituality and forgiveness are often lower in BPD. Among internal-medicine outpatients, people with more borderline symptoms reported lower spiritual well-being (Sansone et al., 2012), and in a sample of 307, lower forgiveness on many measures (Sansone et al., 2013).

Forgiveness grows with recovery. Over 20 years, patients with BPD began about 70% lower than comparison patients in acceptance and forgiveness, but these increased over time, and recovered patients reported about three times the levels of non-recovered patients (Zanarini et al., 2020).

Intervention study. Among 40 adults in outpatient DBT, a **forgiveness module** increased forgiveness and reduced attachment insecurity and psychiatric symptoms, gains kept at 6 weeks; the authors call for a randomized trial (Sandage et al., 2015). Clinicians are also advised to assess and sensitively engage spiritual and religious experiences within standard BPD care (Unruh, 2025).

Honest summary for teaching:

1. DBT and other structured therapies, with good general psychiatric management, are the evidence base. Faith integration supports them; it does not replace them.
2. Forgiveness and acceptance tend to grow as people with BPD recover, and a forgiveness module within DBT showed promising early results.
3. No randomized trial of Christian-integrated treatment for BPD exists yet. Religious experiences can steady or destabilize these clients, so assess them carefully.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within DBT-informed, structured care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for core beliefs that drive abandonment fears, rage, and self-harm, often in spiritual form: "God will leave me like everyone else," "I am too bad to be loved," "I can never be forgiven" (Sansone et al., 2013).
- **Challenge (clinical).** Validate the pain, then test the evidence for all-or-nothing beliefs about self and others; practice skills before trying to change long-held beliefs (Linehan et al., 2006).
- **Replace (pastoral).** Offer Scripture of God's permanent acceptance and of forgiveness:
  - "Though my father and mother forsake me, the LORD will receive me." (Psalm 27:10). For the fear of abandonment.
  - "…for God has said: 'Never will I leave you, never will I forsake you.'" (Hebrews 13:5)
  - "Accept one another, then, just as Christ accepted you, in order to bring glory to God." (Romans 15:7). Acceptance received, then given.
  - "Bear with one another and forgive any complaint you may have against someone else. Forgive as the Lord forgave you." (Colossians 3:13). Forgiveness as a gradual process, not a demand to reconcile with someone unsafe (Sandage et al., 2015).
- **Faith community (clinical).** Help the church offer steady, consistent relationships with clear, kind limits. Unpredictable closeness and rejection can worsen BPD symptoms.

- **Ministers:** keep healthy boundaries, take every mention of suicide or self-harm seriously, and refer to a qualified clinician.

6. Quick classroom takeaways

- BPD involves 5 or more of 9 criteria marked by instability in emotion, relationships, and identity, with impulsivity and self-harm. Suicide risk is high; assess it at every stage.
- DBT and structured care work; standard CBT alone is less clear; medication does not treat BPD itself. Most patients remit over time.
- Forgiveness and acceptance grow with recovery. Replace "everyone will leave me" with the truth that God will never forsake His children.

Sources note

All 18 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Bamelis, L. L. M., Evers, S. M. A. A., Spinhoven, P., & Arntz, A. (2014). Results of a multicenter randomized controlled trial of the clinical effectiveness of schema therapy for personality disorders. *American Journal of Psychiatry*, *171*(3), 305–322. https://doi.org/10.1176/appi.ajp.2013.12040518

Cristea, I. A., Gentili, C., Cotet, C. D., Palomba, D., Barbui, C., & Cuijpers, P. (2017). Efficacy of psychotherapies for borderline personality disorder: A systematic review and meta-analysis. *JAMA Psychiatry*, *74*(4), 319–328. https://doi.org/10.1001/jamapsychiatry.2016.4287

Davidson, K., Norrie, J., Tyrer, P., Gumley, A., Tata, P., Murray, H., & Palmer, S. (2006). The effectiveness of cognitive behavior therapy for borderline personality disorder: Results from the Borderline Personality Disorder Study of Cognitive Therapy (BOSCOT) trial. *Journal of Personality Disorders*, *20*(5), 450–465. https://doi.org/10.1521/pedi.2006.20.5.450

Gunderson, J. G., Herpertz, S. C., Skodol, A. E., Torgersen, S., & Zanarini, M. C. (2018). Borderline personality disorder. *Nature Reviews Disease Primers*, *4*, 18029. https://doi.org/10.1038/nrdp.2018.29

Linehan, M. M., Comtois, K. A., Murray, A. M., Brown, M. Z., Gallop, R. J., Heard, H. L., Korslund, K. E., Tutek, D. A., Reynolds, S. K., & Lindenboim, N. (2006). Two-year randomized controlled trial and follow-up of dialectical behavior therapy vs therapy by experts for suicidal behaviors and borderline personality disorder. *Archives of General Psychiatry*, *63*(7), 757–766. https://doi.org/10.1001/archpsyc.63.7.757

McMain, S. F., Links, P. S., Gnam, W. H., Guimond, T., Cardish, R. J., Korman, L., & Streiner, D. L. (2009). A randomized trial of dialectical behavior therapy versus general psychiatric management for borderline personality disorder. *American Journal of Psychiatry*, *166*(12), 1365–1374. https://doi.org/10.1176/appi.ajp.2009.09010039

Pompili, M., Girardi, P., Ruberto, A., & Tatarelli, R. (2005). Suicide in borderline personality disorder: A meta-analysis. *Nordic Journal of Psychiatry*, *59*(5), 319–324. https://doi.org/10.1080/08039480500320025

Sandage, S. J., Long, B., Moen, R., Jankowski, P. J., Worthington, E. L., Wade, N. G., & Rye, M. S. (2015). Forgiveness in the treatment of borderline personality disorder: A quasi-experimental study. *Journal of Clinical Psychology*, *71*(7), 625–640. https://doi.org/10.1002/jclp.22185

Sansone, R. A., Kelley, A. R., & Forbis, J. S. (2012). Religion/spirituality status and borderline personality symptomatology among outpatients in an internal medicine clinic. *International Journal of Psychiatry in Clinical Practice*, *16*(1), 48–52. https://doi.org/10.3109/13651501.2011.605956

Sansone, R. A., Kelley, A. R., & Forbis, J. S. (2013). The relationship between forgiveness and borderline personality symptomatology. *Journal of Religion and Health*, *52*(3), 974–980. https://doi.org/10.1007/s10943-013-9704-3

Stoffers-Winterling, J. M., Storebø, O. J., Pereira Ribeiro, J., Kongerslev, M. T., Völlm, B. A., Mattivi, J. T., Faltinsen, E., Todorovac, A., Jørgensen, M. S., Callesen, H. E., Sales, C. P., Schaug, J. P., Simonsen, E., & Lieb, K. (2022). Pharmacological interventions for people with borderline personality disorder. *Cochrane Database of Systematic Reviews*, *2022*(11), CD012956. https://doi.org/10.1002/14651858.CD012956.pub2

Storebø, O. J., Stoffers-Winterling, J. M., Völlm, B. A., Kongerslev, M. T., Mattivi, J. T., Jørgensen, M. S., Faltinsen, E., Todorovac, A., Sales, C. P., Callesen, H. E., Lieb, K., & Simonsen, E. (2020). Psychological therapies for people with borderline personality disorder. *Cochrane Database of Systematic Reviews*, *2020*(5), CD012955. https://doi.org/10.1002/14651858.CD012955.pub2

Unruh, B. T. (2025). Addressing spiritual and religious experiences in borderline personality disorder with good psychiatric management. *American Journal of Psychotherapy*, *78*(1), 63–69. https://doi.org/10.1176/appi.psychotherapy.20230047

Winsper, C., Bilgin, A., Thompson, A., Marwaha, S., Chanen, A. M., Singh, S. P., Wang, A., & Furtado, V. (2020). The prevalence of personality disorders in the community: A global systematic review and meta-analysis. *The British Journal of Psychiatry*, *216*(2), 69–78. https://doi.org/10.1192/bjp.2019.166

Zanarini, M. C., Vujanovic, A. A., Parachini, E. A., Boulanger, J. L., Frankenburg, F. R., & Hennen, J. (2003). A screening measure for BPD: The McLean Screening Instrument for Borderline Personality Disorder (MSI-BPD). *Journal of Personality Disorders*, *17*(6), 568–573. https://doi.org/10.1521/pedi.17.6.568.25355

Zanarini, M. C., Frankenburg, F. R., Reich, D. B., & Fitzmaurice, G. (2012). Attainment and stability of sustained symptomatic remission and recovery among patients with borderline personality disorder and axis II comparison subjects: A 16-year prospective follow-up study. *American Journal of Psychiatry*, *169*(5), 476–483. https://doi.org/10.1176/appi.ajp.2011.11101550

Zanarini, M. C., Temes, C. M., Frankenburg, F. R., Reich, D. B., & Fitzmaurice, G. M. (2020). Levels of acceptance and forgiveness reported by patients with BPD and personality-disordered comparison subjects over 20 years of prospective follow-up. *Journal of Personality Disorders*, *34*(2), 262–272. https://doi.org/10.1521/pedi_2019_33_395

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